Anastrozole · daily hormone therapy

Take anastrozole once daily continuously. There is no planned week off. The treatment setting determines the intended duration.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Read the row for your treatment setting. Columns show order, not equal lengths of time. Agree the due window, booking team, result reviewer and whom to contact if a check is missing.

Check	Before starting	First weeks	During treatment	Longer-term review	Change / completion
Bloods / symptom review	FBC, renal and liver profile	Review tolerability and medicines	Further tests when clinically indicated	Review symptoms, adherence and results	Follow unresolved abnormalities
ECG / echo	No routine ECG / echo for this medicine alone	Investigate new cardiac symptoms	Further cardiac tests by risk / symptoms	Companion-drug cardiac rules remain separate	Follow abnormalities when indicated
Bone density / DXA	Adjuvant AI: baseline DXA unless already on adjuvant bisphosphonates	Review previous DXA and fracture risk	No cycle-based DXA appointments	Lower-risk AI: reassess in 1–2 years; individualise	Continue bone care according to risk
Early-breast surveillance	Confirm early / curative setting	Set first follow-up mammogram date	Assess new symptoms promptly	Annual mammography of remaining breast tissue	No routine CT / markers if asymptomatic
Advanced-disease response	Document disease sites / baseline assessment	New symptoms can bring assessment forward	Generally every 2–4 months; individualise	Adapt interval and imaging to disease / benefit	Markers supplement, never replace, assessment

THE WHOLE TREATMENT JOURNEY

Anastrozole · daily hormone therapy: treatment and checks

Postmenopausal hormone-sensitive breast cancer

Take anastrozole once daily continuously. There is no planned week off. The treatment setting determines the intended duration.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First weeks	During treatment	Longer-term review	Change / completion
Treatment plan NCCP 00254	Confirm indication and prescription	Start the agreed daily / injection schedule	Continue unless prescriber changes the plan	Review duration, benefit and tolerability	Agree continuation, switch or completion
Bloods / symptom review NCCP 00254	FBC, renal and liver profile	Review tolerability and medicines	Further tests when clinically indicated	Review symptoms, adherence and results	Follow unresolved abnormalities
ECG / echo NCCP 00254	No routine ECG / echo for this medicine alone	Investigate new cardiac symptoms	Further cardiac tests by risk / symptoms	Companion-drug cardiac rules remain separate	Follow abnormalities when indicated
Bone density / DXA NCCP 00254 · AIBL2025	Adjuvant AI: baseline DXA unless already on adjuvant bisphosphonates	Review previous DXA and fracture risk	No cycle-based DXA appointments	Lower-risk AI: reassess in 1–2 years; individualise	Continue bone care according to risk
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Early: radiotherapy ESMO_EBC2024	Review surgery / pathology	Plan radiotherapy if indicated	Coordinate endocrine timing	Follow the early-breast plan	Review late effects when relevant

What each check is for

1 Confirm the plan

Before starting or changing treatment

Sources: NCCP 00254

2 Checks and symptoms

At the agreed review points

Sources: NCCP 00254

3 Review ongoing benefit

At setting-specific decision points

Sources: NCCP 00254

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.